A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

16 April 2026

Dr Margaret Elder
Medical One QV

Dear Dr Elder,

Re: Mr Thomson Ly
DOB: 1998-05-16
Address: 55 Newcombe Drive, ST ALBANS, VIC, 3021

Summary

- **Functional dyspepsia/disorder of gut-brain interaction (DGBI) symptomatically stable on mirtazapine.**
- **Symptomatic improvement noted with high-fibre diet and lifestyle modifications.**
- **Continue mirtazapine 15mg daily with a future view to weaning.**
- **No action required by you**

I had the pleasure of conducting a review with Thomson, who I have not seen for twelve months. Thank you for your updated referral letter. He remains symptomatically stable on his current regimen of mirtazapine 15mg daily.

Thomson reports that his bowel habits are generally regular. While he experiences occasional episodes of constipation or diarrhoea, these are typically associated with dietary indiscretions. He has successfully implemented a high-fibre diet and reduced his meat intake, which he feels has contributed significantly to his symptomatic control. Furthermore, his energy levels have improved substantially, and he no longer reports the fatigue that was a prominent feature of his initial presentation.

From a psychosocial perspective, Thomson has recently resigned from a high-stress employment environment. He currently reports stable mental health and feels that his reduced stress levels have had a positive impact on his gastrointestinal wellbeing.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
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Impression and Plan Thomson's clinical picture remains consistent with a disorder of gut-brain interaction (DGBI), specifically functional dyspepsia. Given his sustained stability over the last year, we discussed the possibility of weaning his mirtazapine. While he eventually wishes to cease the medication, he prefers to continue the current dose for the time being while he focuses on maintaining his exercise and dietary routines.

I have provided Thomson with a new prescription for mirtazapine 15mg. I have advised him that he has my blessing to attempt a gradual wean before our next appointment if he feels ready; however, there is no clinical urgency to do so given the lack of long-term side effects. I have requested to review him again in twelve months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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